Patient Intake and Triage Standard Operating Procedure

MERIDIAN HEALTH PARTNERS ## OPERATIONAL POLICY MANUAL Section 4: Patient Access and Intake Document ID: MHP-SOP-402 Effective Date: 14 January 2025 Applies To: All Intake Coordinators, Front Desk Specialists, Patient Services Representatives, and Triage Nurses across all 14 Meridian Health Partners sites. Author: Sarah Linwood, Operations Policy Lead Approved By: Dr. Marcus Vance, Chief Medical Officer; Elena Rostova, Director of Clinic Operations

1. PURPOSE AND SCOPE

1.1 This Standard Operating Procedure (SOP) defines the mandatory intake protocol for all patient encounters at Meridian Health Partners. Compliance is mandatory for all administrative and clinical staff handling first-contact communications via telephone, web portal, or walk-in.

1.2 Failure to adhere to data capture standards, urgency banding rules, or record-keeping timeframes may result in internal audit penalties and mandatory retraining administered by the Quality Assurance Department.

2. MANDATORY FIELDS CAPTURED ON FIRST CONTACT

2.1 During any first contact with a prospective or returning patient, the Intake Coordinator must capture and record all twelve (12) mandatory data fields within the Meridian Electronic Health Record (EHR) system under the preliminary encounter file:

* Field 1: Full Legal First Name, Middle Name, and Last Name.
* Field 2: Date of Birth (Format: DD/MM/YYYY).
* Field 3: Primary Contact Telephone Number (Mobile or Landline).
* Field 4: Residential Street Address, City, State, and ZIP Code.
* Field 5: Primary Care Provider (PCP) Name and Clinic Affiliation (if known).
* Field 6: Chief Complaint or Reason for Visit (recorded verbatim in the patient's own words).
* Field 7: Insurance Payer Name and Group Number.
* Field 8: Insurance Member Identification Number (subject to Section 4 exceptions).
* Field 9: Emergency Contact Full Name, Relationship to Patient, and Telephone Number.
* Field 10: Preferred Spoken and Written Language.
* Field 11: Government-Issued Photo Identification Type and Number (for verification upon arrival).
* Field 12: Consent to Receive SMS and Voice Notifications regarding appointment reminders.

2.2 If a patient is unable or refuses to provide any of Fields 1 through 7, 9, 10, 11, or 12, the Intake Coordinator must flag the profile as "Incomplete Intake" and immediately escalate to the Site Operations Supervisor, Arthur Pendelton, for secondary contact resolution within 2 business hours.

3. URGENCY BANDING SCHEME AND TRIGGER PHRASES

3.1 Every intake encounter must be assigned an urgency band based strictly on the clinical indicators and trigger phrases presented by the caller. Intake Coordinators must not downgrade an urgency band without clinical consultation from the on-duty Triage Nurse.

3.2 Emergent
* *Target First-Response Time:* Immediately (live transfer or direct emergency dispatch).
* *Mandatory Trigger Phrases:* "Crushing chest pain", "sudden facial drooping", "inability to speak", "sudden loss of vision", "active suicidal ideation with a plan", "severe shortness of breath at rest", "uncontrolled arterial bleeding", "loss of consciousness within the last 15 minutes", or "severe allergic reaction with throat swelling".
* *Action:* The Intake Coordinator must immediately initiate a warm transfer to emergency services (911) while maintaining the line, and simultaneously notify the on-site Lead Triage Nurse, Brenda Gomez, via the facility internal intercom code Red-3.

3.3 Urgent
* *Target First-Response Time:* Within 1 hour.
* *Mandatory Trigger Phrases:* "High fever above 103.5°F", "severe localized abdominal pain rated 8 out of 10 or higher", "sudden onset of moderate-to-severe dehydration", "suspected fracture with visible deformity", "acute laceration requiring multiple sutures", "sudden adverse reaction to a prescribed Meridian medication", or "sudden severe vertigo preventing standing".
* *Action:* The Intake Coordinator must route the call directly to the on-duty Triage Nurse queue. If the queue is unstaffed due to staff breaks, the call rings over to the secondary clinical pool at the Westside Meridian branch.

3.4 Priority
* *Target First-Response Time:* Within 4 hours.
* *Mandatory Trigger Phrases:* "Worsening chronic condition symptoms", "medication refill required within 48 hours for a maintenance drug", "pain rated between 4 and 7 out of 10 lasting more than 48 hours", "mild infection symptoms such as dysuria or localized cellulitis without systemic fever", "post-operative wound redness increasing over 24 hours", or "unresolved suture removal request past the scheduled window".
* *Action:* The Intake Coordinator must log the encounter in the Priority Triage Queue with an automated timestamp. The scheduling desk must review and allocate a clinician slot within the 4-hour window.

3.5 Routine
* *Target First-Response Time:* Within 24 hours.
* *Mandatory Trigger Phrases:* "Annual wellness exam request", "routine physical for employment or sports", "asymptomatic lab or imaging follow-up review", "general specialist consultation referral intake", "vaccination inquiry", or "routine medication adjustment with stable vitals".
* *Action:* The Intake Coordinator schedules the appointment or places a callback ticket into the standard scheduling worklist to be cleared within 24 hours of contact creation.

4. HANDLING PATIENTS WHO DECLINE TO PROVIDE INSURANCE ID

4.1 If a patient declines to provide their Insurance Member Identification Number (Field 8) during first contact, the Intake Coordinator must invoke the following four-step protocol:

* Step 1: Read the Meridian Financial Transparency Notice script (Document Ref: MHP-FIN-12) explaining that verification of coverage prevents unexpected out-of-pocket liabilities.
* Step 2: Offer the named-exception pathways. Patients presenting under the following named exceptions are permitted to defer insurance ID provision without forfeiting their booking slot:
* *Exception A:* Patients registered under the Meridian Indigent Care Program (MICP), verified via the active sliding-fee scale database managed by Financial Counselor Marcus Vance (Junior).
* *Exception B:* Active participants in the State-Funded Emergency Medical Relief Initiative, substantiated by case worker identification.
* *Exception C:* Patients paying entirely via the Meridian Self-Pay Flat Rate Schedule, who have signed the Self-Pay Financial Agreement form (MHP-SP-09).
* Step 3: If the patient does not qualify for Exceptions A, B, or C, and continues to decline to provide the Insurance ID, the Intake Coordinator must classify the account status as "Self-Pay Undesignated" and inform the patient that they will be billed at the maximum un-contracted fee schedule rate for all services rendered.
* Step 4: The Intake Coordinator must require the patient to execute the "Private Pay Financial Acknowledgement Addendum" prior to appointment confirmation. Under no circumstances may a Routine or Priority appointment be blocked from booking solely due to a missing insurance ID, provided the patient acknowledges the self-pay liability terms.

5. RECORD-KEEPING REQUIREMENTS

5.1 All intake records, audio call recordings, chat transcripts, and web portal submissions must be securely archived within the Meridian EHR central database.

5.2 Retention and Audit Deadlines:
* Intake audio recordings must be retained for a minimum of 7 years from the date of creation for adult patients, and 7 years past the age of majority for minor patients.
* Intake audit logs must be reconciled weekly by the Compliance Officer, Chloe Danvers. Any discrepancy between reported first-response times and system timestamps must be reported to the Operations Policy Lead within 48 hours of discovery.
* All intake documentation must be finalized and locked within the EHR no later than 15 minutes following the conclusion of the first contact. Unlocked draft records exceeding 60 minutes from creation trigger an automated alert to the Site Operations Supervisor.